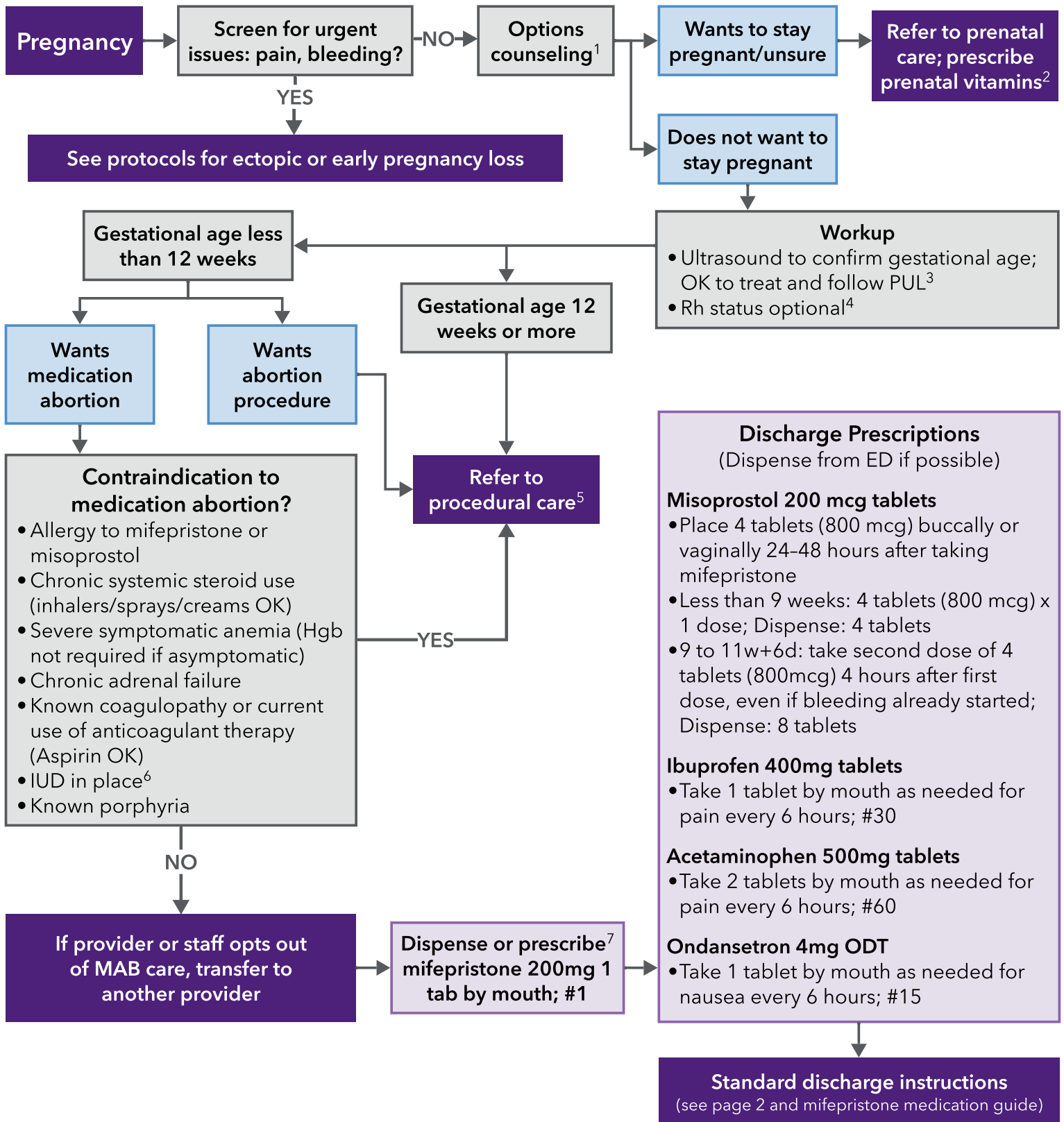


Review protocol with appropriate stakeholders (administration, nursing, OBGYN) as needed prior to implementation.
 Ensure processes in place for seamless handoffs when providers or staff opt out of MAB care.

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The Reproductive Health Hotline (1-844-REPROHH or 1-844-737-7644)

Free, confidential, nationwide, evidence-based hotline for health care providers.

Patient Care

Patient instructions:

- Expect moderate to heavy bleeding, may have clots, seek care if still feel pregnant in a week or if no or only light bleeding occurs.
- Pain can be mild to severe: use NSAID, acetaminophen, heating pads/baths for pain.
- Return to ED if: Soaking 2 pads per hour for 2 hours or more; fever >100.4F for more than 4 hours; uncontrolled pain; feeling ill; or other concerning symptoms.
- MAB is highly effective at 6-11 weeks gestation; may be slightly less effective in very early pregnancy⁸ but do not delay treatment.
- Take an at-home urine pregnancy test 5 weeks after completion of medication; seek care if still positive.

Helpful counseling:

- MAB does not affect future fertility.
- MAB is significantly safer than childbirth.
- MAB has same symptoms of bleeding and cramping as in miscarriage but shorter duration.

Notes for providers:

- Patient must sign manufacturer's treatment agreement and receive medication guide.
- Review expected side effects including bleeding, cramping, low-grade fever, chills, nausea, vomiting, and diarrhea.
- MAB is 96-98% effective with a confirmed intrauterine pregnancy; and 85% effective with pregnancy of unknown location.⁸
- Contraception: Start patch/pill/ring anytime. Discuss other contraception options with PCP or GYN.
- Return to ED if: Soaking 2 pads per hour for 2 hours or more; fever >100.4F for more than 4 hours, uncontrolled pain; feeling ill; or other concerning symptoms.
- If pregnancy of unknown location: ensure hCG follow-up to confirm successful MAB.⁹

Post-treatment:

- Routine in-person follow-up for US or exam is not necessary after uncomplicated MAB.
- Patients can use self-assessment (bleeding, resolution of pregnancy symptoms, and negative urine pregnancy test at 5 weeks) or can be offered clinic visit, quantitative hCG testing, or ultrasound.
- If re-presents to ED and follow-up US done: finding of "uterine debris" is normal. Intervention only needed if retained products of conception (RPOC) are present and patient is symptomatic, including bleeding or significant pain. Taking 2-4 misoprostol tablets can help the uterus evacuate retained tissue and slow bleeding. If no improvement after 1-2 hours, consult GYN. If patients have RPOC but are asymptomatic, expectant management may be appropriate.
- PULs treated by MAB require close hCG follow-up to confirm not an ectopic.⁹

REMS (Risk Evaluation and Mitigation Strategies) 101

- Pharmacy and prescriber must register with one of two authorized mifepristone manufacturers – one prescriber can register for a group.
- Patients must receive and sign a patient agreement specific to that manufacturer and receive their medication guide.
- To start the process to become REMS-certified, click [here](#) for GenBioPro or [here](#) for Danco.

For additional Access Bridge resources, visit bridgetotreatment.org/access-bridge/.

Access Bridge is a program of the Public Health Institute's Bridge Center.

See page 3 for endnotes.

ENDNOTES:

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- 1 [All-Options Talkline](#) 1-888-493-0092; See [options counseling guide](#).
- 2 Folic acid prevents birth defects if taken early in pregnancy (see [CDC](#)).
- 3 If pregnancy of unknown location (PUL): Standard of care allows MAB treatment of PUL. Immediate MAB treatment of PUL is associated with more rapid exclusion of ectopic pregnancy and pregnancy termination but lower abortion efficacy (success rates increase from about 85% in PULs to 97% in confirmed IUPs.). [Goldberg 2022](#).
- 4 Type and screen for potential cross match. Rh testing and RhoGAM administration are not recommended before 12 weeks' gestation for EPL and medical or procedural abortion ([ACOG 2024](#), [SFP 2022](#)). Ectopic pregnancy management should follow standard Rh testing and RhoGAM recommendations. Following institutional guidance is recommended.
- 5 Can refer to [INeedAnA.com](#) or local resource.
- 6 Immediate removal of IUD (by provider or self) in pregnancy is standard of care. If the string is visible in the os, IUDs can be removed easily with ring forceps during a speculum exam; [see here](#) for 58-second how-to video. For desired pregnancies, immediate IUD removal decreases the chance of miscarriage ([ACOG #672 2016](#)). For desired MAB, the IUD must be removed before giving mife/miso. It is best to remove the IUD immediately, but if the patient desires a procedure, the IUD can be removed at the time of the procedural abortion.
- 7 Prescribing to an outpatient pharmacy is an option. Each pharmacy must be REMS-certified and the prescriber must be registered with that pharmacy. For more information, see [REMS 101](#). For list of nearby certified pharmacies stocking mifepristone and misoprostol, visit <https://www.earlyoptionpill.com/how-do-i-get-mifeprex/find-a-certified-pharmacy/> or review GenBioPro list: <https://genbiopro.com/roster/>.
- 8 MAB with pregnancy of unknown location: 85% success rate compared to 97% success rate for confirmed intrauterine pregnancy, [Goldberg 2022](#). Another study showed that early MAB was noninferior to later MAB: [Brandell 2024](#).
- 9 hCG drop of 50% in 48 hours or 80% in 7 days is reassuring for successful MAB ([ACOG Practice Bulletin 225, October 2020](#) and [Goldberg 2022](#)).

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- Repro Health Hotline: <https://reprohh.ucsf.edu>